NBO Visit Cheat Sheet: Step by Step, with the Reasons

A study aid built from your training materials. The Handbook, the official Recording Guidelines, and your mentor's advice come first.

The NBO is an observation, not an assessment. The baby's **state** decides which items you do and when. Follow the baby's lead, and record what you saw **after** the visit, not during it.

Why state matters: every behavior must be interpreted in the context of the baby's state, and a baby can only show some behaviors in some states (for example, shutting out light and sound happens in sleep; looking at a face happens when alert).

1. Read the baby's state first (about 30 seconds)

While the family settles, watch quietly: state, skin color, activity, and any tremors or startles.

State	What you see	What to do
1. Deep sleep	Regular breathing, eyes closed, no eye movements, no movement except startles	Habituation (1 and 2) is possible. Do not wake the baby for other items.
2. Light sleep	Eyes closed with rapid eye movements under the lids, low activity, some sucking movements, irregular breathing	Habituation (1 and 2) is possible. Watch for the baby starting to wake.
3. Drowsy	Eyes open but dull and heavy-lidded, or closed and fluttering; variable activity; slow responses; may wake or sink back to sleep	Wait. Gently support waking (soft voice, unwrapping, holding) or let the baby settle. Not a good state for responsiveness items.
4. Quiet alert	Bright-eyed look, little movement, able to focus on a face or sound	The best window for **responsiveness (9 to 13)**. Use it.
5. Active alert	Eyes open, a lot of movement, brief fussing sounds	Motor items (3 to 8) work well. Settle the baby with comfort steps before trying responsiveness items.
6. Crying	Intense crying that is hard to break through, high motor activity	Pause other items. Observe the crying (14) and use the comfort steps (15). Continue only when calmer.

2. What to do, and what to skip, by state

If…	Then…
Baby is asleep when you arrive	After the introduction and quiet watching, do **habituation to light (1), then sound (2)**. Then uncover gradually if needed. Move on to the awake items as the baby wakes. Do not wake the baby for them.
Baby falls asleep during the visit	**Skip the awake items (3 to 13) for now.** Do not wake the baby to test. Use the sleep: do habituation 1 and 2 if you have not yet. Come back to the awake items if the baby wakes. Note what you could not do.

If…	Then…
Baby is awake and calm at the start	**Skip habituation for now.** It can only be observed in sleep. Do it later if the baby falls asleep (for example after a feed). Start with the awake items; if the baby is in a quiet alert state, use it for 9 to 13 first.
Baby is fussy or active	Settle first with the comfort steps. Fussing is also information for items 14 to 17. Do not push on to responsiveness items until calmer.
Baby is crying	Stop other items. Observe the crying (14). Go through the comfort steps in order (15). Resume later if the baby settles.
Baby is hungry	Invite the parent to feed. Continue afterward. The state will have changed, so check the state again.
Baby shows stress signs	Use the stop, wait, go guide in section 5. Pause or end that item, and comfort the baby with the parent.
An item could not be done	Leave it blank on the Recording Form. Tick only what you administered or observed. Each of the 18 items must appear at least once across your 5 NBOs (use the tracker's coverage grid).

Your question, answered: if the baby falls asleep, **skip the awake items** (motor 3 to 8 and especially responsiveness 9 to 13) and do not wake the baby for them. If the baby is **awake**, skip the **habituation items (1 and 2)**, which can only be done in sleep. Come back to anything skipped if the state changes.

3. The default order (the AMOR reference card, with the official form numbers)

This is the usual flow. The baby's state can change it.

Baby's state	Items	What you can talk about with the parents
Any state	Greeting, goals, quiet observation of the baby's state, the family's story	Warm start; the parent tells you what they have noticed
Sleep	1 Habituation to light · 2 Habituation to sound	Sleep and coping with the environment
Sleep or awake	Uncover and undress (only if needed)	How the baby responds to stress and change; bathing, dressing, changing
Awake	3 Tone · 4 Rooting · 5 Sucking · 6 Hand grasp · 7 Pull-to-sit · 8 Crawl	Need for containment or motor support; feeding cues; safety; positioning (back to sleep, tummy time)
Awake (quiet alert)	9 Face and voice · 10 Visual response to face · 11 Orientation to voice · 12 Orientation to sound (rattle) · 13 Visual tracking (red ball)	Learning the baby's cues; feeding; playing and learning
Crying	14 Crying · 15 Soothability	Crying as communication; comforting the crying baby
All states	16 State regulation · 17 Response to stress · 18 Activity level	Temperament; stimulation threshold

4. Item by item: how, what to watch, why, and when to skip

Rate each item 3, 2, or 1 using the official Recording Guidelines. A rating of 1 describes a pattern; it does not mean something is wrong.

#	Item	How	Watch for	Why it matters (AMOR)	Skip or wait if
1	Habituation to light	Baby **asleep**. Shine a soft light briefly over the closed eyes about every 5 seconds, up to 10 times, or until there is no reaction twice in a row.	Startle, grimace, body movement; how quickly the baby "shuts out" the light	**O** Organization of state. Shows how well the baby can protect sleep. Talk about sleep and coping with the environment.	Skip if awake. Try later if the baby falls asleep.
2	Habituation to sound	Baby **asleep**. Three brisk shakes of the rattle about 30 cm (12 in) above the baby, about every 5 seconds, up to 10 times, or until no response twice in a row.	Same as above	**O** Same as above.	Same as above.
·	Uncover and undress	Place the baby on the back. Remove blankets and clothing as the room, warmth, and the baby's reactions allow.	State change, color change, startle, activity	**A** Autonomic. How the baby copes with change. Talk about bathing, dressing, changing.	Only if needed. Keep the baby warm. The parent may say no.
3	Muscle tone: legs and arms	Gently flex and extend each arm and leg in turn. Release the extension and watch the recoil.	Quality of tone, recoil, evenness	**M** Motor. Talk about containment and motor support.	Needs an awake baby who is calm enough.
4	Rooting	Head at midline. Stroke the corner of the mouth with a gloved finger.	Head turning toward the touch, tongue, mouth opening	**M** Motor. A feeding reflex. Talk about feeding cues.	Awake.
5	Sucking	Place a gloved finger, palm up, in the mouth.	Strength and rhythm of the suck; whether sucking calms the baby	**M** Motor and self-calming. Feeding.	Awake.

#	Item	How	Watch for	Why it matters (AMOR)	Skip or wait if
6	Hand grasp	Place your finger on the palm. Avoid touching the back of the hand.	Strength and quality of the grasp	**M** Motor.	Awake.
7	Shoulder and neck tone (pull-to-sit)	Baby on the back. Hold the hands and pull gently toward sitting. Support the shoulders if needed. Speak at eye level.	Head lag, bringing the head to midline, holding it there	**M** Motor. Head control. Positioning.	Awake.
8	Crawling response	Baby on the tummy, arms bent beside the head.	Crawling movements; lifting and turning the head to free the airway	**M** Motor. Safety, tummy time, back to sleep.	Awake. Stop if the baby is distressed.
9	Response to face and voice	**Quiet alert only.** Hold the baby about 30 cm from your face at about 60 degrees. Talk with an animated face. Move from midline to the sides, then upward. Vary your voice.	Focusing, following smoothly, signs of stress or looking away	**R** Responsiveness (social). Learning the baby's cues, feeding, play.	Skip or wait if asleep, drowsy, or crying. Use comfort and alerting steps; if no quiet alert state comes, move on.
10	Visual response (to face)	Same position. Face and expression only, no voice. Move your face slowly from midline to the sides, then upward.	Focusing and following	**R** Responsiveness.	Same as 9.
11	Orientation to voice	Hold the baby so you are out of sight. Call from one side, or ask the parent to call the baby's name.	Stilling, brightening, turning to find the voice	**R** Responsiveness. Involves the parent.	Same as 9.
12	Orientation to sound (rattle)	You are out of sight. Shake the rattle on one side, then the other. Vary the sound.	Stilling, brightening, turning to find the sound	**R** Responsiveness.	Same as 9.
13	Visual tracking (red ball)	Baby at about 60 degrees. Keep your face and the parent's face out of view. Catch attention at midline. Adjust the distance.	Fixing and following, side to side and up and down	**R** Responsiveness to an object.	Same as 9.
14	Crying	Never try to make the baby cry. Notice how much and how intensely the baby cries through the visit, and which handling brings it on.	Amount, intensity, triggers	**O** Organization of state. Crying is communication.	Observed, not administered. If the baby does not cry, record that.
15	Soothability	Only if the baby cries. First wait a moment to see if the baby settles alone. Then, one step at a time: look and talk; hand on the tummy; hold arms and legs; pick up; rock; swaddle; offer something to suck.	Which step works, how fast, self-soothing	**O** Regulation. Comforting the crying baby.	Only if crying occurs. Otherwise leave it blank.
16	State regulation	Watch throughout the visit.	Which states appear, how smooth or abrupt the changes, how easy the baby is to read	**O** Organization of state.	None. Rate after the visit.
17	Response to stress	Watch throughout.	Color change (mottling, paleness around the mouth), tremors, startles, loss of tone, gagging, breathing changes. Milder: hiccups, sneezes, yawns. The baby's threshold for stimulation.	**A** Autonomic. Stimulation threshold.	None. Rate after the visit.
18	Activity level	Watch throughout.	Amount and quality of movement	**M** Motor and temperament.	None. Rate after the visit.

5. Reading stress: stop, wait, go

Your Module 3 slides use a traffic-light picture (stop, wait, go). The plain-language reading below is a guide; confirm how your mentor uses it.

Signal	What you see	What to do
GO	Calm, steady breathing, relaxed tone, eye contact, smooth changes of state	Continue. The baby is available.
WAIT	Mild stress: hiccups, sneezes, yawns, looking away, a brief startle	Slow down. Pause. Give the baby time to recover. Offer a comfort step. Then try again.
STOP	Stronger stress: color change (mottling, paleness around the mouth), tremors, many startles, gagging, breathing changes, loss of tone, crying that does not settle	Stop that item. Comfort the baby with the parent. Go back to the baby's lead. Say what you see to the parent in plain words.

Safety rule (this is my addition, not from the training slides): if a baby seems to be struggling to breathe, has blue or gray color that does not recover, or is limp or unresponsive, **stop immediately**, support the baby with the parent, and seek urgent medical help (911 or the nearest hospital). Check this with your mentor and the Center's protocol.

6. Talking with the parent, in order

1. Greet warmly; wash your hands; set the space (soft lights, baby at the center, family close, tools within reach).
2. Say who you are: a licensed practitioner in NBO training. Say it is not a test. Describe the goals.
3. Tell them nothing is painful and that most babies cry at some point. Ask how they feel when the baby cries.
4. Ask: "What have you noticed about your baby so far?"
5. Invite the parent in (call the baby's name, help with the light and sound).
6. "Say what you see." Describe the baby's behavior in plain words; celebrate moments of connection; reframe worries kindly.
7. Co-create the review: strengths first, then what is hard right now, then what may help. Say it is "just a snapshot."
8. Fill in the Parent Summary together. The parent keeps the original.

7. Practitioner check (before and during)

Calm? Pause and settle yourself.	**Curious?** Learn, do not only teach.	**Open?** To this family and what is happening.	**Responsive?** To the baby's and the parent's experience now.

8. After the visit, step by step

1. Thank the family. Make sure the parent has the Parent Summary original.

2. Complete the **Recording Form** (initial, gestational age, age only). Tick only the items you administered or observed.
3. Write **Strengths** and **Challenges** in the A-M-O-R format. Use Additional comments for Parent or Practitioner AMOR moments.
4. Photograph the Parent Summary with only the baby's first initial.
5. Write the **Narrative** (500 to 1,000 words) the same day or the next day while it is fresh.
6. Update the **tracker** and the coverage grid. Complete the fidelity self-reflection.
7. File the consents. Store any recording as in the protocol.
8. Send your first 1 or 2 NBOs to your mentor for review before the rest.

9. Narrative outline

Part	What to write
Background	Brief: baby's gestational age and age, how the family came to you, the setting, who was present.
Arrival and state	What you saw first, the baby's state, and what you decided to do because of it.
The visit in order	Item by item: the state, the baby's response, your decisions, and the parent's response.
Connection	Moments between parent and baby (Parent AMOR). Your own state and stance (Practitioner AMOR).
What the baby showed	Strengths, what was hard right now, and what you shared with the parents.
Closing	How the visit ended, what stood out, and what you would do differently.

Optional color-coding in Word: choose one color each for Autonomic, Motor, Organization of state, and Responsiveness, and color the text that relates to each. This shows which systems the baby showed most.

10. Practice before the real visits

- Practice with a doll and with a colleague. Practice bits of the NBO, then the full sequence, then without a parent present.
- Practice with a baby and parents you know. Be honest that you are working on your observation skills.
- Practice reading states: look at the six states and say aloud which you see.